PREOPERATIVE DIAGNOSIS: Severe nasal airway obstruction secondary to deviated nasal septum and bilateral inferior turbinate hypertrophy.

POSTOPERATIVE DIAGNOSIS: Deviated nasal septum with prominent right bony spur; bilateral inferior turbinate mucosal and osseous hypertrophy.

PROCEDURE: Septoplasty and bilateral inferior turbinate submucous resection with radiofrequency ablation.

OPERATIVE TECHNIQUE: The patient was brought to the operating suite and placed under general endotracheal anesthesia. The nasal septum and inferior turbinates were injected submucosally with 1% lidocaine with 1:100,000 epinephrine.

Using a 0-degree endoscope, a standard left-sided Freer hemitransfixion incision was made 2 mm posterior to the caudal septal border. A sharp Freer elevator was used to dissect a pristine subperichondrial flap along the left side of the quadrangular cartilage, extending onto the perpendicular plate of the ethmoid and vomer bone. The cartilage was incised 2 mm posterior to the caudal border, preserving a strict 1.5 cm dorsal and caudal L-strut to prevent nasal tip ptosis and saddle-nose deformity. A contralateral subperiosteal tunnel was created on the right.

A large, sharp bony cartilaginous spur impacting into the right inferior turbinate was isolated and resected using a Jansen-Middleton through-cutting forceps and osteotome. The deviated vomerine crest was excised. The septal cartilage was repositioned into the midline. The mucoperichondrial flaps were reapproximated with a continuous quilting 4-0 plain gut suture on a Keith needle to obliterate dead space and prevent septal hematoma formation.

Attention was turned to the inferior turbinates: A microdebrider turbinate blade was inserted into a stab incision in the anterior head of each turbinate, creating a submucosal pocket. Submucosal stroma was debulked using radiofrequency coblation, followed by lateral outfracture of the turbinate bones using a boies elevator. Symmetrical bilateral nasal airways were established. Soft silicone Doyle nasal splints were placed and secured across the septum with a 2-0 nylon mattress suture.